

Canadian Inborn Errors of Immunity National Registry Policies and Procedures

Table of Contents:

1. Registry purpose
 2. Governance Structure
 3. Funding and Sustainability
 - a. Funding for Registry Support
 4. Ethics, Legal, and Privacy
 5. Data Governance
 - a. Maintaining the Data on the Website
 - b. Accessing CIEINR Data
 - c. CIEINR Data Breach
 - d. Erasure of Personal Data
 - e. Data quality and Assurance
 6. Collaboration with External Organizations
 - a. Processes for Collaboration with External Parties
 - b. Data Transfer to External Parties
 7. Publication Policy
 8. Site Specific Analyses
 9. Exit Policy
-

Registry Purpose, Description and Population

Refer to Study Protocol

Governance Structure

Refer to ToR for CIEINR organization and terms of reference

Funding and Sustainability

Funding Policy for Registry Support

1. Purpose: The purpose of this policy is to establish guidelines for obtaining external funding specifically to support the ongoing operations and management of the Canadian Inborn Errors of Immunity Registry (CIEINR). This includes funding to maintain and expand the registry infrastructure, ensure data management and security, support staffing requirements, and sustain the registry's long-term functionality. Funding requests that are focused solely on supporting the registry's operational needs are eligible under this policy.

2. Eligible Funding Activities: Funding to support the CIEINR registry may be requested for the following activities:

- Registry Infrastructure
 - o Maintenance, upgrades, and development of the registry platform, including IT infrastructure, data storage, security, and hosting services.
 - o Licensing fees for any software tools required for registry operation or integration with other data systems.

- Data security enhancements and ongoing compliance with regulations.
- Data Management and Quality Control
 - Staffing costs for data entry, curation, and quality assurance to ensure accurate and consistent registry information.
 - Development and implementation of protocols for data validation, cleaning, and integrity maintenance.
 - Compliance costs related to data protection, privacy regulations, and patient confidentiality.
- Personnel and Staffing
 - Salaries or stipends for registry staff, including database managers, IT specialists, data analysts, and support personnel.
 - Funding for training programs to ensure staff are up to date on best practices for data management, security, and registry operations.
- Registry Expansion and Sustainability
 - Support for expanding the registry's reach to new institutions or geographic regions.
 - Resources for integrating new data sources, such as clinical systems, laboratory data, or research partnerships, into the registry.
 - Ongoing operational costs to ensure the registry's sustainability over time.
- Public Engagement and Outreach
 - Funding for the creation of materials to promote the registry and encourage institutional participation.
 - Resources for public-facing communications, including reports and updates to stakeholders.

3. Application for Registry Support Funding: Organizations or individuals seeking funding for registry support must submit a formal funding proposal that includes:

- A clear project description outlining the specific registry support needs (e.g., system upgrades, staff support).
- A detailed budget indicating the amount requested and how the funds will be allocated to the relevant registry operations.
- A timeline for fund utilization and expected milestones.
- A justification explaining how the requested funds will enhance the operation, expansion, and sustainability of the CIEINR.
- Finance Manager will oversee the entire funding application process, ensuring that each proposal is reviewed, properly documented, and submitted for evaluation.
- All funding proposals will be evaluated by the CIEINR Steering Committee, which will assess them based on alignment with registry goals, feasibility, and long-term impact.

4. Funding Priorities for Registry Support: Priority for funding to support the CIEINR registry will be given to:

- Critical operational needs such as data security, system upgrades, and essential infrastructure improvements.
- Expansion of the registry to include additional participating institutions or regions, ensuring broader representation and data collection.
- Personnel support to ensure the registry is staffed adequately to manage and maintain high-quality data and infrastructure.
- Sustainability initiatives, including strategies to ensure the long-term financial health of the registry, especially for ongoing maintenance and expansion.

5. Use of Funds: All funds granted for registry support must be used strictly for the purpose outlined in the approved proposal. Any changes to the intended use of the funds require prior approval from the CIEINR Steering Committee. Misuse of funds could result in the revocation of the funding. The Finance Manager will be responsible for monitoring and ensuring that funds are used appropriately and in accordance with the approved proposal. The Finance Manager will also manage the documentation and reporting processes related to fund use.

6. Reporting and Accountability: Recipients of funding must provide regular updates on the use of funds, including:

- Progress reports outlining how the funds have been used, key milestones achieved, and ongoing needs.
- Financial reports documenting expenditures to ensure transparency and accountability in the use of funds.
- A final report summarizing the overall impact of the funding on registry operations, including challenges encountered and solutions implemented.
- Finance Manager will be responsible for overseeing these reports, ensuring timely submission, and reviewing financial documentation to ensure compliance.

7. Ethical and Compliance Considerations: All funding for the registry's operations must adhere to ethical standards, including:

- Data protection compliance: Ensuring patient data privacy and compliance with relevant data protection laws, including the Health Information Act (HIA) and other applicable regulations.
- Transparency and consent: Ensuring the registry's operation respects ethical guidelines and patient consent for the use of their data, in line with ethical standards for health data management.
- Finance Manager will ensure that all funding requests, usage, and reporting align with ethical and legal standards, and will work with the Steering Committee to ensure that funding supports compliance with these considerations.

Ethics, Legal and Privacy

Refer to Study Protocol

Data Governance

Data ownership Registry Data is owned by UCalgary in accordance with its internal policies. The Steering Committee is the data custodian of the CIEINR.

Policy for Maintaining the CIEINR Data on the CINC Website

1. Purpose: There are many reasons why having a patient facing registry website is important: it leads to patient's feeling empowered and encouraged with the work being done, it can increase patient engagement and enrollment into the registry when there is evidence of the meaningful impact of the information being collected, and it can also serve to support research, which can lead to increased advocacy for better patient care. These reasons are just a few examples of why the CIEINR needs to have an organized and transparent patient facing website, that also serves as a point of access to make a

query for interested researchers. The aim of this policy is to describe the maintenance of the CIEINR Data on the CINC Website.

2. Scope: This policy applies to CIEINR Steering committee members, and specifically to the CIEINR Project Manager who will oversee the role of managing the CIEINR Website.

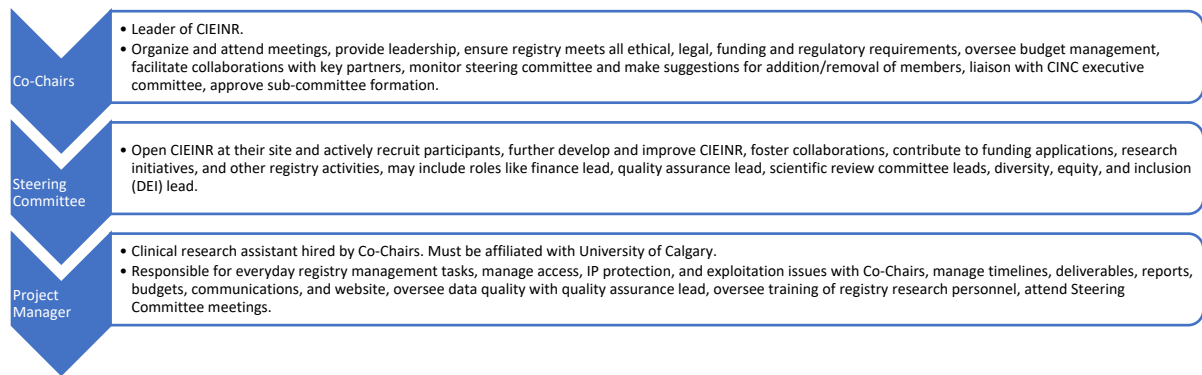
3. Information Included on CIEINR Website:

- Current linkages to other data sources.
- Documentation of key CIEINR characteristics
 - o Data can be filtered by diagnosis and category:
 - Total number of participants
 - Enrollment by diagnosis
 - Enrollment by sex
 - Enrollment by race
 - Enrollment by ethnicity
 - Enrollment by age (years)
 - Enrollment by gene mutation
 - Patient received Ig therapy
 - Patient had transplant
 - Alive (status)
- Current number of sites recruiting patients
- Registry supported publications (in table form, including authors, title, journal, year, volume and pages)
- Registry supported abstracts (in table form, including title, meeting, year, names)
- Registry query form (information to include: name, institution, title/role, email, phone, agreement to abide by CIEINR data use policy and publication policy, purpose of query, overall research question, study population, “how might the information obtained from the CIEINR registry benefit the community?”, “if you know what variables you wish to collect and the specific population to be investigated, list them”. The CIEINR will have an access point for the public and for users of the registry.
 - o Public access point will include a list of studies using CIEINR data, a description of the studies, and a call for participation.
 - o User access point will include an application package, which includes documents, manuals, teaching webinar, list of centres, eCRF and basic form, and research proposal form.

4. Roles and Responsibilities

- The CIEINR website will be managed by the CIEINR project manager.
- The project manager will ensure that the CIEINR website abides by all privacy standards and ethical obligations.
- The project manager will provide a brief CIEINR website update at Steering Committee meetings.
- The project manager will ensure that all registry queries are shared with the Scientific Committee Leads for evaluation in a timely fashion.
- The project manager will work with IT experts to ensure that the CIEINR website is updated with new data quarterly.

5. Decision making Governance Structure of the CIEINR:



Policy for Accessing CIEINR Data

1. Purpose: The primary objective of the CIEINR is to build a registry that provides data regarding IEI to improve knowledge about IEI, increasing advocacy on behalf of IEI patients, and overall, improving patient care. An organized, user friendly approach is necessary for interested researchers, patient advocacy groups, industrial partners, policy makers, etc, to request CIEINR data by submitting research study proposals, that maintains registry integrity and protects the privacy of personal health information. Additionally, it is important that the request for data should be appropriately evaluated to determine whether the appropriate amount of information is available in the CIEINR and whether additional ethics approval is required. The aim of this policy is to describe the process of requesting access to CIEINR data.

2. Scope: This policy applies to the CIEINR Scientific Committee Leads, who will be evaluating and presenting data access requests/study proposals to the steering committee, and to steering committee members who will vote on whether the CIEINR has the data to support the request.

3. Procedures

- Data Access Request
 - 1.1.1 An interested researcher will complete the Registry Query form found on the CIEINR website. This form includes name*, institution*, title/role, email*, phone, agreement to abide by CIEINR data use policy and publication policy*, purpose of query*, overall research question*, study population*, “how might the information obtained from the CIEINR registry benefit the community?”*, and “if you know what variables you wish to collect and the specific population to be investigated, list them” (*Required information).
 - 1.1.2 The registry query will be shared with the Scientific Committee Leads by the Project Manager.
 - 1.1.3 The Scientific Committee Leads will evaluate the data access request, consider if the CIEINR has data to support the request, and will prepare to present the data access request to the steering committee for approval/rejection/further information request.
 - 1.1.4 Within 3 month of submission, the Scientific Review Committee will present to the Steering Committee.
- Presentation to Steering Committee
 - 1.2.1 The Scientific Committee Leads will present the data access request and evidence of ethics approval for the request to the steering committee. After consultation with DEI, a decision will be made regarding whether additional ethics are required.

- 1.2.2 After presentation, the steering committee will discuss whether the CIEINR can support the data access request, considering also whether the CIEINR has the data to support the request and a yes/no vote will take place.
- 1.2.3 If applicable, the CIEINR Steering Committee Disease Specific Working Groups will be consulted whether the data access request is reasonable and data is available to support the presented research question.
- 1.2.4 If applicable, the steering committee may inform the project manager to inform the researcher that additional information is required in order to clarify the request.
- Steering Committee Vote – Yes, sufficient detail
- 1.3.1 If the steering committee votes yes, meaning a minimum of 50% and 5 steering committee members voted yes, the CIEINR has the data to support the request in sufficient detail, all de-identified data pertaining to the request will be collated into a report by bioinformaticians.
 - For industry members, stakeholders, policy makers, and patient advocacy groups, aggregate data will be provided.
- 1.3.2 The bioinformatician's report will be sent to the Steering Committee, providing the opportunity to review and discuss the dataset. The committee must approve its release before the investigator receives any data.
- Steering Committee Vote – Yes, insufficient detail
 - 1.4.1 If the steering committee votes yes, meaning a minimum of 50% and 5 steering committee members voted yes, but the request involves data that is not found in the database in sufficient detail, the Steering Committee may allow the requesting investigator contact site PIs who submitted patients' data matching the search criteria. The investigator would then be responsible for contacting those physicians and establishing a research collaboration to complete acquisition of the data needed for the study.
- Steering Committee Vote – No
 - 1.5.1 If the steering committee votes no, meaning a minimum of 50% and 5 steering committee members voted no, in case e.g. the CIEINR does not have the data to support the request, the project is not feasible, there are concerns regarding ethics or DEI, the project manager will inform the interested research that CIEINR data cannot currently support their request.

Policy for CIEINR Data Breach

1. Purpose: Due to the CIEINR collecting personal health information, it is of the utmost importance that organized data breach procedures are in place so that if a data breach event were to occur, there is a clear approach to containing, assessing and responding to incidents. Additionally, having clear data breach procedures in place assists in establishing and maintaining patient trust, minimize potential harms to patients, and maintain the integrity of the CIEINR. The aim of this policy is to describe procedures that would take place if a data breach event occurred.

2. Scope: This policy applies to the CIEINR Co-chairs, steering committee, and CIEINR IT personnel who will work collaboratively in the event of a CIEINR data breach to contain, assess, and respond to the incident. This policy covers all forms of data that is collected by the CIEINR, including clinical, genetic, treatment, etc.

3. Definitions:

- Data Breach: A data breach occurs when an unauthorized party gains access to sensitive or confidential information, including personal health information.

- Adverse Event: An adverse event is an unanticipated problem involving “risk” to subjects that ultimately result in a breach of confidentiality.

4. Roles and Responsibilities

- Co-Chairs
 - o If a data breach occurs, oversee the utilization of this policy.
 - o If a data breach occurs, provide communication to the public to ensure transparency.
 - o If a data breach occurs and changes to the CIEINR need to be made to ensure another data breach does not occur, work alongside IT personnel in discussing and implementing these changes.
 - o Ensure that ethical obligations and privacy considerations are upheld.
- Steering Committee
 - o If there is a suspected data breach, report this to the rest of the steering committee.
 - o If a data breach occurs, support efforts to contain, assess, and respond to the incident.
- IT Personnel
 - o If a data breach occurs, implement this policy accordingly.
 - o If a data breach occurs, collaborate with the Co-Chairs in containing, assessing, and responding to the incident.
 - o If a data breach occurs, determine the severity of the breach and what information may have been compromised.
 - o Collaborate with the Co-Chairs and Steering Committee to ensure that breach prevention strategies, like staff training on handling personal health information, use of encryption services, audits, etc, are in place.

5. Procedures:

- 1.1 If a suspected data breach has occurred, CIEINR Co-Chairs and Steering Committee must be informed within 24 hours and a data breach incident report will be prepared.
- 1.2 If a data breach has been confirmed, CIEINR Co-Chairs, Steering Committee, and IT personnel must collaborate to secure the CIEINR system to prevent further data loss, and then assess the type of data that has been compromised, number of patients affected, potential harms to the patients affected, and the potential cause of the breach.
- 1.3 If a data breach was confirmed, affected individuals must be notified. This notification should include 1) why the breach occurred, 2) what data was compromised, 3) the risks to the patient associated with the breach, 4) actions taken to address the breach, 5) actions taken to ensure another breach does not happen, 6) the option to withdraw information from the CIEINR.
 - o 1.3.1 If an affected individual chooses to withdraw their information from the CIEINR, please refer to the Policy for the Erasure of Personal Data from CIEINR.
- 1.4 Through the assessment of the breach, if any means of remediation were identified, the Co-Chairs, Steering Committee, and IT personnel should work collaborative to institute these changes.
- 1.5 Thorough documentation of the breach, response actions, and any remediation efforts should be completed and filed in a breach summary binder.

Policy for Erasure of Personal Data from CIEINR

1. Purpose: Having a policy in place for patients to request to have their personal data erased from CIEINR is important as it respects patients’ autonomy, providing them with control over decisions regarding sharing their personal health information, it provides a foundation to build and maintain patient trust, and it can also protect vulnerable patient populations. The aim of this policy is to describe the process of personal data erasure from CIEINR.

2. Scope: This policy applies to the CIEINR project manager, steering committee, and IT personnel who will work collaboratively to ensure that the patient's wishes as it pertains to the sharing of their personal health information is upheld.

3. Procedures:

- 1.1 CIEINR participants have the right to withdraw from the CIEINR at any point without penalty or loss of benefits to which they are otherwise entitled. There is no deadline to withdraw.
- 1.2 If a participant wishes to have their personal data erased from the CIEINR, they must make this request to the enrolling physician who is listed on their consent form.
 - o 1.2.1 It is the responsibility of the enrolling physician to determine whether the patient would like to have all of their personal data removed from the CIEINR, or if their current data can remain with no additional information added.
- 1.3 The enrolling physician would then contact site PI, who would then contact the CIEINR project manager with the patient request.
- 1.4 The CIEINR project manager would present the request to the Steering Committee to ensure Steering Committee members are aware of the request.
- 1.5 The CIEINR project manager would coordinate with IT personnel to fulfill the patient's request.
- 1.6 A report showing evidence of the patient's data being erased from CIEINR would be shared by the project manager to the enrolling physician, who can then provide the report to the patient.

Data Quality and Assurance

1. Purpose: When data is of poor quality—characterized by missing values, inaccuracies, incompleteness, truncation, or outdated information—it may not be suitable for its intended purpose. The inadvertent use of such data can result in inaccurate or misleading reporting and research outcomes. The aim of this policy is to facilitate the communication of data quality concerns to the steering committee and principal site investigators.

2. Scope: This policy applies to CIEINR steering committee and site principal investigators. These personnel are required to comply with all obligations in relation to data management as directed by CIEINR protocol and policies.

2. Procedures:

- Data Entry Responsibility: Site Principal Investigators (PIs) are responsible for ensuring that all data entered into the CIEINR database is accurate, consistent, complete, and submitted in a timely manner. In the event of any questions or concerns regarding data entry, Site PIs must promptly contact the CIEINR Project Manager for clarification and support.
- Oversight of Data Quality: The CIEINR Project Manager shall maintain oversight of data quality across all participating sites.
- Follow-up on Data Inactivity: In cases where no new data has been entered for a period exceeding three years, the Project Manager will contact the respective Site PI to request an updated patient status, including, but not limited to, loss to follow-up or patient death.
- Annual Reporting: The Project Manager will provide an annual report to the CIEINR Steering Committee summarizing the percentage of participants lost to follow-up.
- Prevention of Case Duplication: To minimize the risk of duplicate case registration, Site PIs must inquire during the consent process whether a participant has previously been enrolled in CIEINR. If a participant transitions to a new program, the Site PI will supply the participant with their assigned study identification number to present at the new site.

- The Project Manager will periodically review the REDCap database to identify potential duplicate cases by screening participants with identical dates of birth (month/year) and similar IEI diagnoses. If potential duplicates are identified, the Project Manager will liaise with the respective Site PI to confirm duplication and, if necessary, coordinate the appropriate deletion of duplicate records.

Data dissemination plan

1. Purpose: One important function of the Canadian Inborn Errors of Immunity National (CIEIN) Registry Steering Committee is to facilitate knowledge translation (KT) emerging from research projects utilizing the registry database. The process of KT involves several phases, including synthesis (contextualizing research studies within the larger body of knowledge on the topic), dissemination (sharing results with appropriate audiences and tailoring the message and medium to the audience), data exchange (interaction between knowledge users and researchers, resulting in mutual learning), and application (iterative process by which knowledge is put into practice to improve health and the health system). The ultimate goal of KT with respect to CIEINR is to link research findings from Registry studies to changes in real-world practice and policy, ensure that relevant scientific findings inform clinical and policy decision making and future research, and improve outcomes for individuals with IEI, their families and communities.

2. Principles

- Steering Committee members will engage knowledge users throughout the research process, to ensure that the knowledge is relevant and usable.
- CIEINR Steering Committee members will facilitate KT through collaboration and communication with policymakers, practitioners, patient organizations, patient advisors and other stakeholders.
- Data generated from CIEINR research projects will be communicated to Clinical Immunology Network – Canada (CINC) members, CINC partners, and key stakeholder groups (including patient organizations, patients, funding agencies and government) in a timely and thoughtful fashion.
- Facilitation of rapid data release: Data producers have responsibilities to release data rapidly and to publish initial analyses in a timely manner. Presentation of findings at conferences and publication in scientific journals will be agreed upon by the Registry Steering Committee, in adherence with the publication policy.
- Dissemination of relevant information on research project(s) using the CIEIN Registry through adequate and timely measures, including via the CINC website.
- Whenever possible, patient and/or their representative will be consulted to facilitate optimal KT including data dissemination
- KT activities will be evaluated for efficacy
- The impact of research on people living with IEI should be a key consideration on the data dissemination plan. Research projects will appropriately acknowledge the contribution of patients and their representatives.
- Resources, data and results should be shared among CIEINR research projects and made publicly available to the broader community

Collaboration with External Organizations

Decision Making Governance Structure and Processes for Internal CIEINR Activities and Collaboration with External Parties

1. Purpose: To maintain integrity of the CIEINR, provide transparency to patients regarding how their data may be used, and build collaborative relationships with interested external parties, it is important

that the decision-making governance structures and processes related to the CIEINR are described. The aim of this policy is to describe the decision-making governance structure of the CIEINR, and the process of making decisions both internally and with external parties.

Scope: This policy applies to CIEINR Steering Committee who will be involved in decision making, and specifically to the Project Manager who will be presenting collaboration interests of external parties to the CIEINR Steering Committee.

2. Procedures:

1.1 Internal

- 1.1.1 – CIEINR Co-chairs can make recommendations to the Steering Committee about the addition or removal of Committee members.
- 1.1.2 – CIEINR Co-chairs can suggest and approve the formation of sub-committees as required.
- 1.1.3 – To pass a motion or make a group decision, a minimum of 50% and 5 Steering Committee members must agree.

1.2 External, for Use or Disclosure of Data to External Parties for Academic or Research Purposes or Non-Research Purposes (eg: Regulatory, Health Technology Assessment Agencies, Industry)

- 1.2.1 – A request for collaboration is made via the request form on the CIEINR website.
- 1.2.2 – This request is received by the project manager and is presented at the next Steering Committee meeting.
- 1.2.3 – The Steering Committee will vote on whether collaboration should occur (the project manager is a non-voting member of the meeting). A minimum of 50% and 5 Steering Committee members must agree.
- 1.2.4 – The project manager will inform the interested collaborator on the CIEINR Steering Committee's decision.

Policy for Data Transfer Between CIEINR and External Parties

1. Purpose: Ensuring patient privacy, data integrity, and the maintenance of all ethical and legal obligations are maintained is one of the most important considerations of the CIEINR. This is why having an organized approach to data transfer between CIEINR, and external parties is of the upmost importance. The aim of this policy is to describe the data transfer policy between the CIEINR and external parties, including researchers, collaborators, health authorities, industry partners, etc.

2. Scope: This data applies to all potentially transferable data within the CIEINR. It also applies to all Steering Committee members to ensure that the policy is upheld.

3. Procedures:

- Data transfer must be approved by the Steering Committee after “request to data” form submitted on the website. (see Policy for accessing CIEINR Data). Data transfer must comply with the regulatory compliance described in the Data Transfer Agreement and informed consent. De-identified or merged Registry Data will only be accessed, used, stored, transferred, disclosed or otherwise handled in accordance with the Protocol and the Participant's informed consent form.
- The Project Manager of the CIEINR needs to keep track of data transfers on the data transfer log. The recipient must abide by data transfer agreements, must not try to re-identify de-identified data, must store data securely, limit access to data, report any data breaches, and retain data for only as long as stated in the data transfer agreement. External Party Investigators shall not provide Registry Data to any third party.

Publication Policy

1. Purpose: To ensure appropriate notification, oversight, and acknowledgment for all abstracts, manuscripts, and scholarly outputs using data from the Canadian Inborn Errors of Immunity Registry (CIEINR), in accordance with registry governance and legal agreements.

2. Scope: This policy applies to all abstracts, manuscripts, meeting presentations, and public releases that use CIEINR data, including pooled multicentre analyses.

3. Procedure

- Intent to Publish – Notification: Prior to submitting any abstract or manuscript using CIEINR data, researchers must notify the CIEINR Project Manager by email. The notification must include: A copy of the abstract or manuscript, intended submission date, target journal or scientific meeting
 - Review and Distribution: The CIEINR Project Manager will distribute the submission to the Steering Committee. The CIEINR Manager and Co-Chairs will ensure that all publications are shared with required partners in accordance with applicable legal agreements. Where appropriate, the Steering Committee may provide guidance on data interpretation, scope of conclusions, or other relevant considerations prior to submission. A response will be provided by the Project Manager within two weeks.
 - Consortium Acknowledgment: All publications, abstracts, meeting presentations, and public communications using CIEINR data must include the following acknowledgment: "The Canadian Inborn Errors of Immunity Registry (CIEINR) is supported by [specific funding source or grant number]."
-

Site-Specific Analyses (Single-Centre Data Only):

1. Purpose: To provide oversight for site-specific analyses using single-centre data from Canadian Inborn Errors of Immunity Registry (CIEINR), while supporting investigator-initiated research, ensuring alignment with registry objectives, and preventing duplication or conflict with existing or planned registry projects.

2. Scope: This policy applies to studies using CIEINR data exclusively from a single participating site (i.e., not pooled multicentre registry data). Such studies are not subject to the registry's data access, publication, or authorship policies applicable to multicentre analyses, except where explicitly outlined below.

3. Procedure

- Submission and Review: All proposed site-specific analyses must be submitted to the Steering Committee for review and approval prior to study initiation. Submissions are intended to ensure alignment with registry objectives, adherence to ethical considerations, and avoidance of overlap with existing or planned projects. The Registry Manager will circulate the proposal to the Steering Committee for electronic review.
- Approval Process: Approval decisions will be communicated within two weeks of submission. Approval requires a minimum of five Steering Committee members and at least 50% of votes in favor.
- Authorship: The Registry Co-Chairs must be included as co-authors on all resulting abstracts and manuscripts in recognition of their oversight, governance, and stewardship of the registry infrastructure.

- Acknowledgment: All abstracts and manuscripts arising from site-specific analyses must include the required Consortium Acknowledgment.
 - Publication Tracking: Final versions of all abstracts and manuscripts must be submitted to the Steering Committee for tracking and record-keeping purposes.
-

Exit policy/Legacy Planning

1. Purpose: The purpose of this SOP is to describe the processes that will be followed in the event that the Registry is terminated, or there are significant changes to its operation (Legacy Planning), for example transferring data to another appropriate location, or the destruction of its data. This could happen in the event of leadership changes, funding challenges, or completion of data collection.

2. Scope: This SOP pertains to the CIEINR Steering Committee Chair(s), who will determine whether the Legacy Plan should be enacted, as well as all CIEINR personnel who will execute the processes herein.

3. Procedures

- 1.1 Preparing: When there is a triggering event requiring data exit policy to be enacted (e.g. change in leadership or leadership location, loss of funding), the CIEINR will initiate the following legacy planning phase.
 - 1.1.1 Appoint a member of its personnel to coordinate the following activities.
 - 1.1.2 Implement a detailed current state assessment of the data. This will require provision of detailed information by the prospective transferor and/or access to assess all the aspects below and will include:
 - Number of registered patients
 - Format of data
 - How and where data are stored
 - Details on previous use of data
 - Format for coding and level of data privacy (coding system, anonymization or de-identification details)
 - Consent status (informed consent details or basis for a waiver of consent)
 - 1.1.3 Determine valuation of scope and scale of data including:
 - Common versus rare disease types
 - Large or specialized population cohorts
 - Data collection format
 - Age of data
 - Amount and extent of available demographic and outcome data
 - Nature of consent (i.e., the specific commitments around research use and future storage of data, and including the type of consent in terms of broad versus specialized use allowed)
- 1.2 Developing the Legacy Plan: The CIEINR Steering Committee Chairs, in collaboration with the host institution and its other Registry leadership/governance roles/committees will oversee the following steps to develop the Legacy Plan.
 - 1.2.1 Using the current state assessment (1.1.2), assemble a list of Registries/research groups that may be interested in acquiring the data. Consider:
 - Feasibility and logistics of transferring data.
 - Current ethical, social and financial information along with any legal

- implications related to transferring the data
 - Funding and resource requirements to transfer the data.
- 1.2.2 Formulate a rationale and recommended decision to destroy data, or to keep/transfer data (the decision may include a combination of both options).
- 1.2.3 Consider any stakeholders that need to be included in the planning and/or implementing phases of a legacy plan (e.g., participants, institutional department heads, Research Ethics Boards, funding bodies, third party contractors) and develop an appropriate Communications strategy. In particular, consider if there is a duty to ensure that participants maintain the ability to withdraw their data from future research use and if they need to be notified of the decision.
- 1.2.4 The CIEINR Steering Committee Chairs and the host institution's leadership will make the decision to destroy or keep/transfer data on basis of the information obtained from the above assessment including consultations with appropriate stakeholders. Detail the rationale, formal decision and Communications strategy in a Legacy Plan document.
- 1.3 Execution: Identify all tasks required for the execution phase (see Appendix A) and assign to appropriate biobank personnel to complete.
 - 1.3.1 Confirm timelines for all tasks.
 - 1.3.2 Implement communication plan to relevant stakeholders.
 - 1.3.3 If data are to be destroyed, maintain accurate records of the process and store in final location when completed.
 - 1.3.4 If data are to be transferred, maintain accurate data transfer records and store in final location when completed. Include (if applicable):
 - Material Transfer Agreements
 - Other identified agreements
 - Who the data were sent to
 - Where the data were transferred (i.e., existing Registry, archived data in network drives, or database servers)
 - Ethical approvals at time of transfer

Example list of tasks required for Execution Phase

Retrieval		Quality Assessment		Transfer	
Process	Personnel	Process	Personnel	Process	Personnel
Identifying which data belongs to the Registry and the relations between data	Data manager	Record QA/QC data results	Data manager	Migrating the data using defined migration rules	Data manager
(a) Fully understanding the data sources		Identify quality issues in data		a) Start with a small sample to test	
(b) Understand the history, structure, meaning of source data		Validate/test the data			
Identifying required system/software				Follow up and maintenance	

a) what software is needed to process that data (e.g. format of the database) b) hardware					
Planning the migration of the data a) Which parts can/should be migrated/where manual input is easier b) Specifications for mapping the data					

Modified from:

https://files.biobanking.org/brc/sops/01/eng/01.007_Legacy_Planning_e1.0.pdf
